a) Evolutionary

- Once an organism has produced viable offspring its body is no longer needed. It then ages and dies.
- Certain genes may be useful in early life but detrimental in later life.
- Late acting genes have not been removed because they act after reproduction.

b) Physiological

- Programmed theories maintain that ageing occurs due to intrinsic timing mechanisms and signals. eg. Genetic timers.
- Stochastic theories maintain that ageing occurs as the result of chance or accidental events. eg. free radical damage.



Nodal Union Ministry For Senior Citizens

1. National policy on older persons.
2. The implementation strategy.
3. Inter – ministerial committee.
4. Training of manpower for care of Senior citizens.
5. Concession and facilities given to senior citizens.

7.5 MENTAL HEALTH

Changes in cognitive ability, excessive forget fullness and moodswings are not a part of normal aging. Changes in mental status may be related to many factors such alteration in diet and fluid and electrolyte balance. Therefore health professionals must recognize, assess, refer, collaborate, treat and support older adults exhibit noticeable changes in intellect of affect.

7.5.1 Some Of The Common Problems

- Depression
- Acute confusional state (Delirium)
- Anxiety disorders
- Cognitive impairment and dementia
- Late life delusional disorder
- Alcoholism
- Personality disorder
- Obsessive compulsive disorder
- Drug and substance abuse.
- Sleep and rest pattern
- Coping abilities

7.5.2 Dementia

Dementia and acute confusional state usually are the result of organic diseases characterised by impairment of intellectual functions. E.g..aphasia, aphaxia



Clinical Manifestations

- Language impairment
- Apraxia (difficulty in performing skilled tasks)
- Agnosia (inability to recognise objects)
- Impaired executive function (poor performance, poor judgement and planning)

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